

Arthritis and Pregnancy in the UK

Planning a pregnancy with arthritis? UK guide to medication safety, disease management during pregnancy, NHS preconception services, and breastfeeding with arthritis.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Advisory Panel** (HCPC-registered physiotherapy & rheumatology nursing input) · 2026-03-17 · livingwitharthritis.org.uk

If you're living with arthritis and planning a pregnancy, or are already pregnant, there's good news: **many women with arthritis have healthy pregnancies**. However, careful planning with your rheumatologist is essential, particularly around medication management.

Before You Conceive

Ideally, speak to your rheumatology team **3-6 months before trying to conceive**:

- Some arthritis medications must be stopped well before conception
 - Your disease should be well-controlled before pregnancy — active inflammation increases risks
 - Ask your GP or hospital for a preconception counselling appointment
 - Start folic acid (400 micrograms daily) at least 3 months before conception
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Medication Safety



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Generally Safe During Pregnancy

- Hydroxychloroquine — considered safe and often continued throughout
- Sulfasalazine — generally safe but requires folic acid supplementation
- Low-dose prednisolone — may be used for flare management
- Paracetamol — first-line pain relief in pregnancy

Must Be Stopped Before Pregnancy

- Methotrexate — must be stopped at least 3 months before conception (men and women)
- Leflunomide — requires a washout period

- NSAIDs (ibuprofen, naproxen) — avoid after 20 weeks; discuss with doctor before 20 weeks

Discuss with Your Rheumatologist

- Biologics — some are safer than others during pregnancy; many are stopped in the third trimester
 - JAK inhibitors — generally stopped before conception
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During Pregnancy

Many women with inflammatory arthritis experience improvement during pregnancy, particularly in the second and third trimesters. However:

- Some women experience flares, especially in the first trimester
 - Regular monitoring with both rheumatology and obstetric teams is important
 - Stay as active as possible — swimming and walking are excellent during pregnancy
 - Physiotherapy can help with joint pain and posture changes
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After Birth



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- Flares are common in the first 3-6 months postpartum — plan medication restart with your team
- Breastfeeding is possible with many arthritis medications — check each one individually
- Ask for help at home — managing a newborn with arthritis requires support
- Contact your rheumatology team early if symptoms worsen

UK Resources

- UK Teratology Information Service (UKTIS) — bumps.org for medication safety data
- NHS preconception clinics — ask your rheumatologist for a referral
- NRAS — leaflets on RA and pregnancy

With proper planning and support from your healthcare team, arthritis need not prevent you from having a healthy pregnancy and baby.

Q&A

Question: What is the key thing to know about arthritis and pregnancy in the uk?

Short answer: Managing arthritis during pregnancy requires careful planning with your rheumatologist. Some medications like methotrexate must be stopped months before conception, while others like hydroxychloroquine are considered safe. Folic acid (400 micrograms daily) should be started at least three months prior. Regular monitoring throughout pregnancy and planning for potential postpartum flares are crucial for a healthy outcome.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Versus Arthritis: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.